RISK MANAGEMENT PLAN (RMP)

Executive Overview • Training draft based on the supplied synthetic Workora evidence pack

Product selected for this draft: Losartan
Reference file: Synthetic Core Data Sheet v1.0
Safety data window: 1 January–24 August 2026

1. Purpose and scope

This executive overview outlines proposed safety specifications, pharmacovigilance activities, and risk-minimisation measures for a hypothetical newly launched losartan product. The supplied reference marks fatigue as “Unexpected”; therefore, fatigue is treated here as a potential safety concern requiring validation and further evaluation—not as a confirmed adverse drug reaction or established safety signal. This is an educational draft and is not a regulator-approved RMP.

2. Summary of the supplied data

The aggregate workbook contains 320 synthetic reports across eight products. It contains 38 reports for losartan; the recorded reaction is fatigue in these rows. Of the losartan reports, 38 are marked unexpected in the aggregate file and 3 are marked serious. These are report counts only: the workbook does not provide exposure denominators, validated causality, or enough clinical detail to estimate incidence or establish causation.

Dataset item	Losartan extract
Reports in dataset	38
Reaction recorded	Fatigue
Reference expectedness	Unexpected (Synthetic Core Data Sheet v1.0)
Seriousness flag: Yes	3
Observation period	1 Jan–24 Aug 2026

3. Safety specifications

Important identified risks: No product-specific identified risk can be confirmed from these synthetic files alone. Known risks must be verified against the current approved local product information before finalisation.

Potential risk / topic for evaluation: Fatigue reported with losartan and labelled unexpected in the supplied reference. Review case narratives, time to onset, dose, concomitant medicines, underlying conditions, dechallenge/rechallenge, alternative causes, seriousness, and clinical outcome. Consider signal validation and prioritisation by a qualified safety team.

Missing information: Safety in populations or circumstances not adequately represented in the evidence pack; pregnancy/lactation, paediatric use, significant renal or hepatic impairment, and long-term use should be assessed against the product label and actual evidence. These are assessment topics, not assertions that risks are present.

4. Pharmacovigilance plan

Routine activities

• Maintain a system for intake, validation, coding, medical review, follow-up, and timely reporting of individual case safety reports (ICSRs).
• Monitor spontaneous reports, literature, and other available safety sources; reconcile cases and maintain traceable records.
• Conduct periodic aggregate review and benefit–risk evaluation according to applicable local requirements and the product’s data lock point / reporting schedule.
• Maintain training, quality checks, audit trails, and documented escalation of potential signals.

Additional activities proposed for the fatigue topic

• Perform targeted medical review of fatigue cases and request missing clinical information where appropriate.
• Review seriousness, temporal association, dechallenge/rechallenge, concomitant medicines, and plausible alternative causes.
• Trend cases over time and assess whether a formal signal validation is warranted; document rationale and decisions.
• Reassess the need for additional studies or enhanced monitoring only if the cumulative evidence and applicable authority guidance support them.

5. Risk-minimisation measures

Routine measures: Ensure the approved prescribing information and patient leaflet are accurate and accessible; counsel patients to use the medicine as prescribed and to report persistent or troublesome fatigue or other suspected adverse effects to a healthcare professional. Healthcare professionals should assess severe, sudden, or function-limiting symptoms and consider other causes and concomitant medicines.

Additional measures: No additional educational material or patient alert card is justified by the supplied synthetic data alone. If further evaluation confirms a clinically meaningful risk, develop targeted healthcare-professional guidance or patient materials, obtain required review/approval, and evaluate their effectiveness.

6. Effectiveness monitoring and review

Review the quality and completeness of fatigue reports, proportion with adequate follow-up, seriousness and outcome patterns, time-to-onset information, and whether the safety topic changes the product's benefit–risk assessment. Document review outcomes and update the RMP when new evidence, labelling changes, or regulatory requirements warrant it.

7. Data limitations and quality checks

The supplied workbook is synthetic and should not be interpreted as real-world evidence. Counts do not represent incidence or risk. The aggregate dataset contains fields that require case-level validation (including dose, route, indication, and expectedness consistency); verify these against source records before drawing conclusions. The product reference is a training reference, not a substitute for current approved product information. This draft assumes Losartan is the intended product because fatigue is the only reaction labelled "Unexpected" in the reference sheet; confirm the intended product with the task owner if needed.

8. Suggested final document structure

Executive summary; product and scope; safety specifications (identified risks, potential risks, missing information); pharmacovigilance plan (routine/additional activities); risk-minimisation measures (routine/additional); effectiveness monitoring; references and version control.

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